

Patient Name	: Anjana Thakur	Barcode	: E7956430	
Age/Gender	: 46Y 2M 24D /Female	Sample Collected On	: 25/Apr/2026 10:15AM	
Order Id	: 17961740570	Sample Received On	: 25/Apr/2026 04:18PM	
Referred By	: Self	Report Generated On	: 25/Apr/2026 05:21PM	
Customer Since	: 25/Apr/2026	Sample Temperature	: Maintained ✓	
Sample Type	: EDTA Plasma	Report Status	: Final Report	

DEPARTMENT OF BIOCHEMISTRY HBA1C

Test Name	Value	Unit	Bio. Ref Interval
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HbA1c - Glycosylated Hemoglobin

Hba1c (Glycosylated Hemoglobin)	4.60	%	4.2 - 5.7
Method: HPLC			
Machine: TOSOH G11			
Average Estimated Glucose	85.32	mg/dl	
Method: Calculated			

INTERPRETATION:

AS PER AMERICAN DIABETES ASSOCIATION (ADA): REFERENCE GROUP

Non diabetic
At Risk (Prediabetes)
Diagnosing Diabetes

GLYCOSYLATED HEMOGLOBIN (HBA1c) in %

<5.7

5.7 – 6.4

>= 6.5

Age > 19 Years

Goals of Therapy: < 7.0

Actions Suggested: >8.0

Age < 19 Years

Goal of therapy: <7.5

Therapeutic goals for glycemic control

REMARKS :

- HbA1c is used for monitoring diabetic control. It reflects the mean plasma glucose over three months
- HbA1c may be falsely low in diabetics with hemolytic disease. In these individuals a plasma fructosamine level may be used which evaluates diabetes over 15 days.
- Inappropriately low HbA1c values may be reported due to hemolysis, recent blood transfusion, acute blood loss, hypertriglyceridemia, chronic liver disease. Drugs like dapsone, ribavirin, antiretroviral drugs, trimethoprim, may also cause interference with estimation of HbA1c, causing falsely low values.
- HbA1c may be increased in patients with polycythemia or post-splenectomy.
- Inappropriately higher values of HbA1c may be caused due to iron deficiency, vitamin B12 deficiency, alcohol intake, uremia, hyperbilirubinemia and large doses of aspirin.
- Trends in HbA1c are a better indicator of diabetic control than a solitary test.
- Any sample with >15% HbA1c should be suspected of having a hemoglobin variant, especially in a non-diabetic patient. Similarly, below 4% should prompt additional studies to determine the possible presence of variant hemoglobin.
- HbA1c target in pregnancy is to attain level <6 % .
- HbA1c target in paediatric age group is to attain level < 7.5 %.

Method : Ion-exchange high-performance liquid chromatography (HPLC).

Reference : American Diabetes Associations. Standards of Medical Care in Diabetes 2023

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Order Id	: 17961740570	Sample Received On	: 25/Apr/2026 04:26PM	
Referred By	: Self	Report Generated On	: 25/Apr/2026 05:10PM	
Customer Since	: 25/Apr/2026	Sample Temperature	: Maintained ✓	
Sample Type	: Sodium Fluoride	Report Status	: Final Report	

DEPARTMENT OF BIOCHEMISTRY

Test Name	Value	Unit	Bio. Ref Interval
Glucose-Random			
GLUCOSE, RANDOM	110	mg/dl	70 - 140
Method: Hexokinase			
Machine: BECKMAN COULTER DxC 700 AU			


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Order Id : 17961740570
Referred By : Self
Customer Since : 25/Apr/2026
Sample Type : Serum

Barcode : E7956430
Sample Collected On : 25/Apr/2026 10:15AM
Sample Received On : 25/Apr/2026 05:24PM
Report Generated On : 25/Apr/2026 06:45PM
Sample Temperature : Maintained ✓
Report Status : Final Report

DEPARTMENT OF BIOCHEMISTRY

Test Name	Value	Unit	Bio. Ref Interval
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Liver Function Test (LFT)

Serum Bilirubin, (Total) Method: Diazonium Ion Machine: BECKMAN COULTER AU 5800	0.43	mg/dl	0.3 - 1.2
Serum Bilirubin, (Direct) Method: Diazotization Machine: BECKMAN COULTER AU 5800	0.08	mg/dl	0 - 0.2
Serum Bilirubin, (Indirect) Method: Calculated	0.35	mg/dl	0.0 - 0.8
Aspartate Aminotransferase (AST/SGOT) Method: UV withP5P Machine: BECKMAN COULTER AU 5800	19.60	U/L	3 - 35
Alanine Aminotransferase (ALT/SGPT) Method: UV without P5P - IFCC Ref. Proc., Calibrated Machine: BECKMAN COULTER AU 5800	17.4	U/L	3 - 35
Alkaline Phosphatase (ALP) Method: IFCC AMP Buffer Machine: BECKMAN COULTER AU 5800	72.60	U/L	33-98
Gamma Glutamyl Transferase (GGT) Method: G-glutamyl-carboxy- nitroanalide-IFCC Machine: BECKMAN COULTER AU 5800	19.7	U/L	5- 38
Serum Total Protein Method: Biuret Machine: BECKMAN COULTER AU 5800	7.27	gm/dl	6.6 - 8.3
Serum Albumin Method: Bromo Cresol Green(BCG) Machine: BECKMAN COULTER AU 5800	3.81	g/dl	3.5 - 5.2
Serum Globulin Method: Calculated	3.46	gm/dl	3.0 - 4.2
Albumin/Globulin Ratio Method: Calculated	1.10	Ratio	1.2 - 2.5
SGOT/SGPT Ratio Method: Calculated	1.13	Ratio	0.7 - 1.4

Bilirubin is a yellowish pigment found in bile and is a breakdown product of normal heme catabolism. Elevated levels results from increased bilirubin production (eg hemolysis and ineffective erythropoiesis); decreased bilirubin excretion (eg; obstruction and hepatitis); and abnormal bilirubin metabolism (eg; hereditary and neonatal jaundice). Conjugated (direct) bilirubin is elevated more than unconjugated (indirect) bilirubin in viral hepatitis; drug reactions, alcoholic liver disease conjugated (direct) bilirubin is also elevated more than unconjugated (indirect) bilirubin when there is some kind of blockage of the bile ducts like in Gallstones getting into the bile ducts tumors & Scarring of the bile ducts. Increased unconjugated (indirect) bilirubin may be a result of hemolytic or pernicious anemia, transfusion reaction & a common metabolic condition termed

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Referred By	: Self	Report Generated On	: 25/Apr/2026 06:45PM
Customer Since	: 25/Apr/2026	Sample Temperature	: Maintained ✓
Sample Type	: SERUM	Report Status	: Final Report

DEPARTMENT OF BIOCHEMISTRY

Test Name	Value	Unit	Bio. Ref Interval
Iron study			
Serum Iron	65.3	ug/dl	60 - 180
Method: TPTZ			
Machine: BECKMAN COULTER AU 5800			
UIBC	309.70	ug/dl	155 - 355
Method: Nitroso-PSAP			
Machine: BECKMAN COULTER AU 5800			
Serum Total Iron Binding Capacity (TIBC)	375	µg/dl	250 - 400
Method: FE+UIBC (saturation with iron)			
Transferrin Saturation %	17.41	%	15 - 50
Method: Calculated			

Iron participates in a variety of vital processes in the body varying from cellular oxidative mechanisms to the transport and delivery of oxygen to body cells. It is a constituent of the oxygen-carrying chromoproteins, haemoglobin and myoglobin, as well as various enzymes, such as cytochrome oxidase and peroxidases.

Serum iron may be increased in hemolytic, megaloblastic and aplastic anemias, and in hemochromatosis acute leukemia, lead poisoning, pyridoxine deficiency, thalassemia, excessive iron therapy, and after repeated transfusions. Drugs causing increased serum iron include chloramphenicol, cisplatin, estrogens (including oral contraceptives), ethanol, iron dextran, and methotrexate. Iron can be decreased in iron-deficiency anemia, acute and chronic infections, carcinoma, nephrotic syndrome hypothyroidism, in protein- calorie malnutrition and after surgery. Diurnal variation is seen in serum iron levels with normal values obtained in the midmorning, low values in midafternoon and very low values near midnight.

TIBC measures the blood's capacity to bind iron with transferrin (TRF). Estrogens and oral contraceptives increase TIBC levels. Asparaginase, chloramphenicol, corticotropin, cortisone, and testosterone decrease the TIBC levels.

Transferrin is the primary plasma iron transport protein, which binds iron strongly at physiological pH. Transferrin is generally only 25% to 30% saturated with iron. The additional amount of iron that can be bound is the unsaturated iron-binding capacity (UIBC). Transferrin saturation represents the number of iron-binding sites that are occupied. It is a better index of iron stores than serum iron alone. Transferrin saturation is decreased in iron deficiency anemia (usually <10% in established deficiency).


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Referred By	: Self	Report Generated On	: 25/Apr/2026 07:00PM	
Customer Since	: 25/Apr/2026	Sample Temperature	: Maintained ✓	
Sample Type	: Serum	Report Status	: Final Report	

DEPARTMENT OF BIOCHEMISTRY

Test Name	Value	Unit	Bio. Ref Interval
Lipid Profile			
Total Cholesterol	173.8	mg/dL	Desirable : <200
Method: Cholesterol Oxidase, Esterase, Peroxidase			Borderline: 200-239
Machine: BECKMAN COULTER AU 5800			High : >=240
Serum Triglycerides	165.4	mg/dl	Desirable : <150
Method: Enzymatic			Borderline high : 150-199
Machine: BECKMAN COULTER AU 5800			High : 200-499
			Very high : >= 500
Serum HDL Cholesterol	44.1	mg/dl	40 - 60
Method: Direct measure, immunoinhibition			
Machine: BECKMAN COULTER AU 5800			
LDL Cholesterol Calculated	96.57	mg/dl	Optimal : <100
Method: Calculated			near /above Optimal:100 - 129
			Borderline High: 130- 159
			High : 160 - 189
			Very High :>=190
VLDL Cholesterol Calculated	33.09	mg/dl	<30
Method: Calculated			
Total CHOL / HDL Cholesterol Ratio	3.94	Ratio	3.30 - 4.40
Method: Calculated			
LDL / HDL Cholesterol Ratio	2.19	Ratio	Desirable/Low Risk: 0.5-3.0
Method: Calculated			Line/Moderate Risk: 3.0-6.0
			Elevated/High Risk: >6.0
HDL / LDL Cholesterol Ratio	0.46	Ratio	Optimal->0.4
Method: Calculated			Moderate-0.4 to 0.3
			High-<0.3
Non-HDL Cholesterol	129.7	mg/dl	0.0 - 160.0
Method: Calculated			

Dyslipidemia is a disorder of fat or lipoprotein metabolism in the body and includes lipoprotein overproduction or deficiency. Dyslipidemias means increase in the level of one or more of the following: Total Cholesterol, low density lipoprotein (LDL) and/or triglyceride concentrations.

Dyslipidemia also includes a decrease in the "good" cholesterol or high-density lipoprotein (HDL) concentration in the blood. Cholesterol is a steroid carried in the bloodstream as lipoprotein, necessary for cell membrane functioning and as a precursor to bile acids, progesterone, vitamin D, estrogens, glucocorticoids and mineralocorticoids.

HDL is termed "good cholesterol" because its levels are inversely related to the risk of Coronary heart disease. LDL cholesterol is termed the "bad cholesterol" and their increased levels are associated with increased risk of atherosclerosis and coronary


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Sample Type : Serum

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DEPARTMENT OF BIOCHEMISTRY

Test Name	Value	Unit	Bio. Ref Interval
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Kidney Function Test

Serum Creatinine Method: Jaffes Kinetic Machine: BECKMAN COULTER AU 5800	0.82	mg/dl	0.4 - 0.9
GFR, ESTIMATED Method: Calculated (CKD-EPI)	89.15	mL/min/1.73m ²	
Serum Uric Acid Method: Uricase Machine: BECKMAN COULTER AU 5800	4.6	mg/dl	2.6-6.0
Blood Urea Method: Urease Machine: BECKMAN COULTER AU 5800	11.97	mg/dl	17 - 43
Blood Urea Nitrogen (BUN) Method: Calculated	5.6	mg/dl	8-20
Bun/Creatinine Ratio Method: Calculated	6.82	Ratio	
Urea/Creatinine Ratio Machine: BECKMAN COULTER AU 5800	14.60		

The kidneys play a vital role in the excretion of waste products and toxins such as urea, creatinine and uric acid, regulation of extracellular fluid volume, serum osmolality and electrolyte concentrations, as well as the production of hormones like erythropoietin and 1,25 dihydroxy vitamin D and renin. Assessment of renal function is important in the management of patients with kidney disease or pathologies affecting renal function. Tests of renal function have utility in identifying the presence of renal disease, monitoring the response of kidneys to treatment, and determining the progression of renal disease.

Urea is synthesized in the liver as the final product of protein and amino acid metabolism. Urea synthesis is therefore dependent on daily protein intake and endogenous protein metabolism.

Creatinine is a metabolic product of creatine and phosphocreatine, which are both found almost exclusively in muscle.

EGFR-National kidney Disease Education program recommends the use of MDRD equation to predict/ estimate GFR in adults (>=20 years) with Chronic kidney disease. Calculated by serum creatinine may be less accurate due to certain factor like race muscle mass, diet and certain drugs in such case EGFR should be calculated using serum cystatin C.

Age (Years)	Male ML/min/1.73m ²	Female ML/min/1.73m ²
17-24	93-131	—
25-34	78-146	—
35-44	74-138	65-123
45-54	74-129	58-110
55-64	69-122	50-110
65-74	61-114	46-105
75-84	52-102	48-85

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Patient Name : Anjana Thakur
Age/Gender : 46Y 2M 24D /Female
Order Id : 17961740570
Referred By : Self
Customer Since : 25/Apr/2026
Sample Type : Whole Blood EDTA

Barcode : E7956430
Sample Collected On : 25/Apr/2026 10:15AM
Sample Received On : 25/Apr/2026 04:18PM
Report Generated On : 25/Apr/2026 05:31PM
Sample Temperature : Maintained ✓
Report Status : Final Report

DEPARTMENT OF HAEMATOLOGY

Test Name	Value	Unit	Bio. Ref Interval
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Complete Blood Count

Haemoglobin (HB) Method: Photometric Measurement Machine: BECKMAN COULTER DxH800	8.6	g/dL	12.0-15.0
Total Leucocyte Count (TLC) Method: Coulter Principle Machine: BECKMAN COULTER DxH800	5.3	10 ³ /uL	4.0-10.0
Hematocrit (PCV) Method: Calculated Machine: BECKMAN COULTER DxH800	31	%	36.0-46.0
Red Blood Cell Count (RBC) Method: Coulter Principle Machine: BECKMAN COULTER DxH800	4.30	10 ⁶ /μl	3.80-4.80
Mean Corp Volume (MCV) Method: Derived from RBC Histogram Machine: BECKMAN COULTER DxH800	72.6	fL	83.0-101.0
Mean Corp Hb (MCH) Method: Calculated Machine: BECKMAN COULTER DxH800	20.2	pg	27.0-32.0
Mean Corp Hb Conc (MCHC) Method: Calculated Machine: BECKMAN COULTER DxH800	27.8	g/dL	31.5-34.5
RDW - CV Method: Derived from RBC Histogram Machine: BECKMAN COULTER DxH800	37.6	%	11.6-14.0
RDW - SD Method: Derived from RBC Histogram Machine: BECKMAN COULTER DxH800	90.60	fL	39.0-46.0
Mentzer Index Method: Calculated	16.88	Ratio	
RDWI Method: Calculated	634.83	Ratio	
Green and king index Method: Calculated	230	Ratio	

Differential Leucocyte Count

Neutrophils Method: VCS Technology Machine: BECKMAN COULTER DxH800	69.7	%	40 - 80
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Sample Temperature : Maintained ✓
Report Status : Final Report

DEPARTMENT OF HAEMATOLOGY

Test Name	Value	Unit	Bio. Ref Interval
Lymphocytes Method: VCS Technology Machine: BECKMAN COULTER DxH800	21.5	%	20-40
Monocytes Method: VCS Technology Machine: BECKMAN COULTER DxH800	4.8	%	02 - 10
Eosinophils Method: VCS Technology Machine: BECKMAN COULTER DxH800	3.1	%	01 - 06
Basophils Method: VCS Technology Machine: BECKMAN COULTER DxH800	0.9	%	00 - 02
Absolute Leucocyte Count			
Absolute Neutrophil Count (ANC) Method: Calculated Machine: BECKMAN COULTER DxH800	3.69	10 ³ /uL	2.0-7.0
Absolute Lymphocyte Count (ALC) Method: Calculated Machine: BECKMAN COULTER DxH800	1.14	10 ³ /uL	1.0-3.0
Absolute Monocyte Count Method: Calculated Machine: BECKMAN COULTER DxH800	0.25	10 ³ /uL	0.2-1.0
Absolute Eosinophil Count (AEC) Method: Calculated Machine: BECKMAN COULTER DxH800	0.16	10 ³ /uL	0.02-0.5
Absolute Basophil Count Method: Calculated Machine: BECKMAN COULTER DxH800	0.05	10 ³ /uL	0.02 - 0.10
Platelet Count(PLT) Method: Coulter Principle Machine: BECKMAN COULTER DxH800	237	10 ³ /μl	150-410
MPV Method: Derived from PLT Histogram Machine: BECKMAN COULTER DxH800	9.6	fL	7 - 9

The International Council for Standardization in Haematology (ICSH) recommends reporting of absolute counts of various WBC subsets for clinical decision making. This test has been performed on a fully automated 5 part differential cell counter which counts over 10,000 WBCs to derive differential counts. A complete blood count is a blood panel that gives information about the cells in a patient's blood, such as the cell count for each cell type and the concentrations of Hemoglobin and platelets. The cells that circulate in the bloodstream are generally divided into three types: white blood cells (leukocytes), red blood cells (erythrocytes), and platelets (thrombocytes). Abnormally high or low counts may be physiological or may indicate disease conditions, and hence need to be interpreted clinically.

The Mentzer index is used to differentiate iron deficiency anaemia beta thalassemia trait. If a CBC indicates microcytic anaemia, these are two of the most likely

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Sample Type : Serum

Barcode : E7956430
Sample Collected On : 25/Apr/2026 10:15AM
Sample Received On : 25/Apr/2026 05:24PM
Report Generated On : 25/Apr/2026 09:30PM
Sample Temperature : Maintained ✓
Report Status : Final Report

DEPARTMENT OF IMMUNOLOGY

Test Name	Value	Unit	Bio. Ref Interval
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Vitamin B12

VITAMIN B12	515	pg/ml	211 - 912
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Method: CLIA

Vitamin B12 is a coenzyme that is involved in two very important metabolic functions vital to normal cell growth and DNA synthesis: 1) the synthesis of methionine, and 2) the conversion of methylmalonyl CoA to succinyl CoA. Deficiency of this vitamin can lead to megaloblastic anemia and ultimately to severe neurological problems. Also causes macrocytic anemia, glossitis, peripheral neuropathy, weakness, hyperreflexia, ataxia, loss of proprioception, poor coordination, and affective behavioral changes. A significant increase in RBC MCV may be an important indicator of vitamin B12 deficiency.

Patients taking vitamin B12 supplementation may have misleading results. A normal serum concentration of B12 does not rule out tissue deficiency of vitamin B12. The most sensitive test for B12 deficiency at the cellular level is the assay for MMA. If clinical symptoms suggest deficiency, measurement of MMA and homocysteine should be considered, even if serum B12 concentrations are normal.

Vitamin D, 25-Hydroxy

VITAMIN D (25 - OH VITAMIN D)	11.29	ng/ml	30 - 100
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Method: CLIA

VITAMIN D STATUS	VITAMIN D 25 HYDROXY (ng/mL)
DEFICIENCY	<20
INSUFFICIENCY	20 – <30
SUFFICIENCY	30 – 100
TOXICITY	>100

Vitamin D is a lipid-soluble steroid hormone that is produced in the skin through the action of sunlight or is obtained from dietary sources. The role of vitamin D in maintaining homeostasis of calcium and phosphorus is well established.

The assay measures both D2 (Ergocalciferol) and D3 (Cholecalciferol) metabolites of vitamin D. Vitamin D status is best determined by measurement of 25 hydroxy vitamin D, as it is the major circulating form and has longer half life (2-3 weeks) than 1,25 Dihydroxy vitamin D (5-8 hrs)

The reference ranges discussed in the preceding are related to total 25-OHD; as long as the combined total is 30 ng/mL or more, the patient has sufficient vitamin D.

Levels needed to prevent rickets and osteomalacia (15 ng/mL) are lower than those that dramatically suppress parathyroid hormone levels (20–30 ng/mL). In turn, those levels are lower than levels needed to optimize intestinal calcium absorption (34 ng/mL). Neuromuscular peak performance is associated with levels approximately 38 ng/mL.


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Customer Since	: 25/Apr/2026	Sample Temperature	: Maintained ✓	
Sample Type	: Serum	Report Status	: Final Report	

DEPARTMENT OF IMMUNOLOGY

Test Name	Value	Unit	Bio. Ref Interval
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Thyroid Profile (Total T3, T4, TSH)

Tri-Iodothyronine (T3, Total)	1.18	ng/ml	0.87 - 1.78
Method: CLIA			
Machine: BECKMAN COULTER DxI800			
Thyroxine (T4, Total)	6.27	ug/dl	5.48-14.28
Method: CLIA			
Machine: BECKMAN COULTER DxI800			
Thyroid Stimulating Hormone (TSH)-Ultrasensitive	2.116	μIU/mL	0.38 - 5.33
Method: CLIA			
Machine: BECKMAN COULTER DxI800			

Pregnancy interval	Bio Ref Range for TSH in uIU/ml (As per American Thyroid Association)
First trimester	0.1 - 2.5
Second trimester	0.2 - 3.0
Third trimester	0.3 - 3.0

Healthians recommends that the following potential sources of variation should be considered while interpreting thyroid hormone results:

1. Thyroid hormones undergo rhythmic variation within the body this is called circadian variation in TSH secretion: Peak levels are seen between 2-4 AM. Minimum levels seen between 6-10 AM. This variation may be as much as 50% thus, influence of sampling time needs to be considered for clinical interpretation.
2. Circulating forms of T3 and T4 are mostly reversibly bound with Thyroxine binding globulins (TBG), and to a lesser extent with albumin and Thyroid binding Pre-Albumin. Thus the conditions in which TBG and protein levels alter such as chronic liver disorders, pregnancy, excess of estrogens, androgens, anabolic steroids and glucocorticoids may cause misleading total T3, total T4 and TSH interpretations.
3. Total T3 and T4 levels are seen to have physiological rise during pregnancy and in patients on steroid treatment.
4. T4 may be normal even in the presence of hyperthyroidism under the following conditions : T3 thyrotoxicosis, Hypoproteinemia related reduced binding, during intake of certain drugs (eg Phenytoin, Salicylates etc)
5. Neonates and infants have higher levels of T4 due to increased concentration of TBG
6. TSH levels may be normal in central hypothyroidism, recent rapid correction of hypothyroidism or hyperthyroidism, pregnancy, phenytoin therapy etc.
7. TSH values of <0.03 uIU/mL must be clinically correlated to evaluate the presence of a rare TSH variant in certain individuals which is undetectable by conventional methods.
8. Presence of Autoimmune disorders may lead to spurious results of thyroid hormones.
9. Various drugs influence the levels of thyroid hormones such as L-Dopa, Lithium, Glucocorticoids, Phenytoin etc.
10. Healthians recommends evaluation of unbound fractions, that is free T3 (fT3) and free T4 (fT4) for clinic-pathologic correlation, as these are the metabolically active forms.

*** End Of Report ***

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